الإختبار النهائى للفصل العاشر للعام الدراسى ٢٠٢٠ /٢٠٢١ (مادة طب الاطفال)

التاريخ : ٢٠٢١\٦\٢٦

الوقت:ساعة

الورقة الثانية (امتحان العملى وأعمال السنة) فى مادة طب الاطفال

الرجاء الاجابة على أسئلة الجزء العملى فى Section (A) و أعمال السنة فى Section (B)

أولا- امتحان العملى (٢٥ درجة) الرجاء الاجابة على جميع الأسئلة

Case study

11 months old infant presents to outpatient's clinic because delayed sitting and repeated chest infection. History revealed exclusive breast feeding. On examination he has large box shaped head, wide anterior fontanel, and palpable swellings at wrist joint and costochondral junctions

1- What is the probable diagnosis

a) Protein energy malnutrition b) Marasmus

c) Rickets d) Pediatric immunodeficiency

2- The most common cause of this disorder in such case is-

a) Magnesium deficiency b) Calcium deficiency

c) Vitamin D deficiency d) Phosphorus deficiency

3-What is the treatment of this condition

a) Oral vitamin D 1500-5000IU \ DAY for 4 weeks

b) Shock therapy 600000 IU I M single dose

c) Calcium if there is convulsion

d) All of above

4- Which one of the following is considered a good source of the defective nutrient in this infant (choose only one answer)

a) Cow`s milk

b) Fortified infant formula

c) Breast milk

d) fruits and vegetables

5- Other clinical manifestation of this disorder include (choose only one answer)

a) Flag sign

b) Bowing of the legs

c) Auscultation of pansystolic murmur on heart

d) Cracky paint dermatosis

Case study

A 3 day old full term neonate, presents with yellowish discoloration of his eyes and skin. The mother had O +ve blood group and the father had A +ve blood group. On examination, he is well with good suckling. His laboratory report shows hemoglobin 9 gm/dl, total bilirubin 24 mg/dl mainly indirect bilirubin

6 - What is the most probable diagnosis.

a) Neonatal sepsis

b) Neonatal jaundice

c) Hypercarotenemia

d) Hepatitis

7 - The indirect bilirubin is much elevated in this case, what is the . possible explanation:

a) Hypothyroidism

b) Chronic hemolytic anemia as thalassemia

c) ABO incompatibility

d) Inborn error of metabolism

8 - The best first line of treatment.

a) Restrict breast feeding

b) Encourage breast feeding and phototherapy

c) Exchange transfusion

d)Antibiotics

9 - **If the level of bilirubin continues to rise in spite of starting treatment, the best option for treatment could be**

a) Stop breast feeding and start IV fluid

b) Exchange transfusion

c) Increase dose of antibiotics

d) Give packed RBCs

10 - If this infant develops convulsions, what is the suspected complication

a) Intracranial hemorrhage

b) Lactic acidosis

c) Kernicterus

d) CNS infection

Case study:

A 10 month old boy came to the outpatient's clinic complaining of progressive weight loss, decreased appetite and repeated attacks of diarrhea. History revealed exclusive insufficient breast feeding. On examination the patient's weight was 4 Kg with marked muscle wasting, loss of Subcutaneous fat and abdominal distension with no lower limb edema

11- What is the most likely diagnosis

a- Rickets

b- Marasmus

c- Kwashiorkor

12- What is the most appropriate aetiology for this case

a- Pyloric stenosis

b- Recurrent gastroenteritis

c- Recurrent bronchiolitis

13- What is the caloric treatment of this case (To catch weight)

a- 120 kcl/kg/day

b- 250 kcl/ kg/day

c- 200 kcl/ kg/day

14- Feeding supplement with the weaning food should be given

a- Every two hours

b- Every six hours

c- Every four hour

15- If patient on breast feeding:

a) Continue .breast feeding

b) Stop breast feeding

c) Start lactose free formula

Case study

A 6 month old infant was presented in the outpatient clinic, with history of recurrent attacks of cyanosis observed with crying, and recurrent attacks of chest infections. He was very irritable and was markedly distressed with tachycardia and tachypnea. On auscultation a pansystolic murmur was heard on left side of chest wall.

16- What is the probable cause of this audible murmur

a) Fallot tetralogy

b) Ventricular septal defect

c)TGA

d) Pneumonia

17- The best diagnostic procedure to detect cardiac defect

a) CXR

b) Clinical Signs

c) ECG

d) Echocardiography

18- What is the most possible serious complication? (Choose only one answer)

a) Convulsions

b) Bronchiectasis

c) Infective endocarditis

d) Bleeding tendency

19- Treatment of heart failure include (one answer)

a) Increase intake of fluids

b) Avoidance of inotropes as Digoxin

c) Large frequent healthy meals

d) Preload and afterload reduction

20. Signs

a) Che

c) Con

Case stud

A male c days dura with feve On exam Temperat patches, r

21) What

a- Acute le

b- Idiopath

c- Aplastic

22) The ne

a- Com

b- Bon

c- Liver

23) The mo

a- Drug inta

b- Infection

c) Idiopathic

24) The mos

- IV immun

- Bone mar

What are possible complications of a VSD?

Complications of an untreated VSD include:

- Lung problems
- Heart failure
- Irregular heart rhythms (arrhythmias)
- Heart valve problems
- Poor growth and development

20. Signs of digitalis toxicity is:

a) Chest infection

b) Arrhythemia

c) Convulsion

d) None of the above

Case study

A male child 4 years old presented with epistaxis & red skin eruption of 7 days duration. No bleeding from other body orifices & was not associated with fever or bone pain or abdominal distension. No history of drug intake. On examination: Blood pressure 90/60 mm hg, pulse 100 beat /min, Temperature. 36.9 C, generalized purpuric eruption with ecchymotic patches, no pallor, no jaundice, no palpable organs or lymph nodes

acute

21) What is your provisional diagnosis?

a- Acute leukemia

b- Idiopathic thrombocytopenic purupra (ITP)

c- Aplastic anemia

22) The next step in diagnosis is to do?

a- Complete blood count.

b- Bone marrow transplantation

c- Liver function test

23) The most appropriate aetiology of the case is?

a- Drug intake

b- Infection

c- Idiopathic

24) The most appropriate treatment of the case ?

a- IV immunoglobulin

b- Bone marrow transplantation

c- IV antibiotics

25) The most common outcome of that condition

a- Mostly complete improvement

b- Mostly will develop bone marrow failure

c- Most cases end by dea

Management

ITP has no cure, and relapses may occur years after seemingly successful medical or surgical management. [6] Most children with acute ITP do not require treatment, and the condition resolves spontaneously. [7, 8]

ثانيا- امتحان أعمال السنة (١٠ درجات)

1) If patient with nephrotic syndrome has significant edema, diuretics & Aldosterone antagonists (Fursemide, Spironolactone) can be used:

A) True B) False

2) Convulsions in acute diarrhea can be due to:

A.Hypercalcemia B.Hyperkalemia
C.Hypokalemia D.Hypernatremia

3) Malabsorption (pale bulky offensive stool with abdominal distension) is characteristic of which of the following:

A.Shigellosis B.Salmonellosis
C.Amebiasis D.Giardiasis

4) Which of the following is an indication of intravenous rehydration?

A.Diarrhea more than 10 times/day B.The child is eager to drink
C.Severe abdominal distension D.Vomiting 4 times/day

5) Symptomatic treatment of pancytopenia includes all of the following except:

A- Red blood cell transfusion.
B- Platelet transfusion.
C- Antibiotics.
D- All of above.

6) Immunization against poliomyelitis can be given by:

A. Oral route
B. Intramuscular route
C. Both of them
D. None of them

❖ Indications for I.V. fluids:

1) Shock or coma
2) Failure of oral or N.G. rehydration
3) Severe dehydration
4) Persistent vomiting (>4 times /hour)
5) Anuria
6) Gastric dilatation
7) Convulsions
8) Severe abdominal distension

7) Which vaccine should be protected from light:

A. Polio vaccine

B. Pertussis vaccine

C. BCG vaccine

D. Typhoid vaccine

8) The adverse reaction which makes us use only DT subsequently and not DPT is:

A. Convulsions

B. Fever

C. Irritability

D. Rash

9) Sabin vaccine is characterized by:

A. It can be used in immune-compromised individuals

B. It induces systemic and local intestinal immunity

C. It is an inactivated formalin-killed virus vaccine

D. It is monovalent vaccine

10) A 12-hour-old baby is found on the postnatal ward to be grunting and dusky. He was born at term by normal vaginal delivery (NVD) after an uneventful pregnancy with normal antenatal scans. On examination he has increased work of breathing with subcostal and intercostal recession, nasal flaring and grunting. He has a systolic murmur and 2-cm liver edge palpable. Capillary refill time is 3 s and he is alert. His heart rate (HR) is 190 and respiratory rate (RR) 68. Oxygen saturations are measured as 72%. He is transferred to the neonatal intensive care unit (NICU) where his perfusion becomes poor and respiratory effort reduced. What is the immediate next step in the management of this baby?

a) High-flow oxygen

b) Inhaled nitric oxide

c) Prostaglandin infusion

d) Inotropic support

Good Luck